



Safety Planning (Mental Health) Policy and Procedure

Preamble

Consumer Participation

The Rockingham Peel Group (RKPG) recognises and fully supports the need for the patient and/or their primary carer to¹:

- Know and exercise their healthcare rights.
- Be engaged in their healthcare.
- Have access to information about treatment options.
- Participate in treatment decisions.
- Have access to information about agreed treatment plans.

Scope of Practice

It is the responsibility of each health care provider to ensure all patient care and therapeutic interventions they provide are within their individual scope of practice. Refer to the [RKPG Guidelines to Writing for Clinical Policy](#) for further information about Scope of Practice.

Background

The purpose of this policy and procedure is to minimise preventable harm to consumers who access mental health services, by assessing, identifying and safety planning with consumers, carers, and family in a collaborative understanding manner to address identified risks. Mental health consumers expect clinical care which is supported by shared decision making and shared responsibility for safety. Care provided must promote contemporary mental health principles being:

- Recovery focused.
- Person-centered
- Trauma informed.
- Culturally competent.
- Developmentally.

Procedure

Assessing risk

- There are several risks and/or potential risks which must be assessed by mental health clinicians. These include but are not limited to:
 - * Suicide and self-harm.
 - * Violence and aggression.
 - * Sexual safety concerns.
 - * Deterioration in mental health.
 - * Deterioration in physical health.
 - * Capacity and ability to engage with treatment or care.
 - * Potential to become a missing person or leave care without negotiation with clinicians.

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Jun 2024	Revisions	NA	National Standards 1, 5, 6
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- * Exploitation.
- * Reputational damage.
- * Homelessness.
- The most appropriate clinician should undertake collaborative risk assessments with consumers and, where possible, carers and family at the following times:
 - * Activation/admission.
 - * Presentation to Emergency Department (ED).
 - * Contact with triage.
 - * Throughout episode of care.
 - * Prior to and on return of leave episodes.
 - * Prior to change of setting or transfer of care.
 - * Discharge.
 - * Following a change in clinical status.
- Clinicians should also refer to: [RKPG Intake Meeting, Case Allocation and Case Review \(Mental Health\) Policy](#) and the RKPG (PaRK) Mental Health Standardised Clinical Documentation and Process Standard Operating Procedure for further clarifications on documentation standards and risk assessment responsibilities.
- All risk assessments should be documented on the Mental Health, Risk Assessment and Management Plan (RAMP) SMHMR905 within [webPSOLIS](#).
- Mimidi park clinicians outside of the SSCD pathway requirements may thereafter utilise the 'Progressive Risk' assessment. This may be used prior to leave on return from leave, or if there's a change in clinical status or as part of shift documentation.
- In assessing risk staff should seek to gather information from all available sources when and where appropriate. This includes from carers, family and next of kin (NOK), clinicians must seek permission and do so in line with confidentiality and consumer rights.
- [South Metropolitan Health Service \(SMHS\) Rights of patients in mental health](#)

Safety planning for identified risks

- To minimise preventable harm and promote safety, risk/s and potential risk/s identified in risk assessment needs to be addressed in safety plans. Initial safety planning can occur on the RAMP within the "Risk Management Plan" section. Successive safety planning should utilise the [Consumer Safety Plan](#).
- The Consumer Safety Plan must:
 - * Be developed in collaboration with the consumer.
 - * Involve where appropriate and available engagement with carers and family.
 - * Be linked to and incorporated into current treatment support and discharge plans.
 - * Revised and updated regularly, ideally at scheduled reviews and when there's a change in clinical status. Schedule time for review of safety plans.
 - * Be provided to consumers, carers, and family where appropriate.
 - * Be revised and updated regularly throughout the episode of care, when there is a change in clinical presentation and when care is being transferred.
 - * Recognise specific triggers and circumstances which may compromise safety.

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- * Include strategies to reduce risk and enhance safety:
 - o Collaborating with the consumer, carer, and family about interventions which are likely to work.
 - o Liaising and discussing with other health care professionals.
 - o Linking safety planning to identified consumer strengths and recovery goals.
- * Identify actions to be taken in a crisis and indicating who is responsible for the action.
- * Include culturally safe practices and approaches.

Safety planning for people who may be suicidal

Health professionals must:

- Conduct early assessment and timely reassessment of consumers who may be suicidal. Refer to:
 - * [Department Of Health \(DoH\) Principles and Best Practice for the Care Of People Who May Be Suicidal](#)
 - * [RKPG Care of Person Who May Be Suicidal \(Acute\) Policy](#).
- Document information for safety planning and share with the consumer, carer and family.
- Ensure assertive 7-day follow-up processes for persons discharged from inpatient care who may be at risk of being suicidal.

Safety planning for people who may be at risk of becoming violent or aggressive

Health professionals must:

- Conduct early assessment and timely reassessment of consumers who may be at risk of becoming violent or aggressive. Refer to:
 - * [DoH Principles and Best Practice for the Care Of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)
 - * [RKPG Acutely Aroused Patient Clinical Practice Standard \(CPS\)](#)
 - * [RKPG Restraint and Seclusion: Reducing Restrictive Interventions CPS](#).
- Document information regarding safety planning for the consumer's care, which must be shared with the consumer, their carers, and family, and other services/agencies, unless there are exceptional circumstances.
- Following an incident, review the consumer's safety plan and update as required.

Ensuring the sexual safety of mental health consumers

- Health professionals must recognise factors that impact the sexual safety of mental health consumers including:
 - * Physical environment.
 - * Use of gender specific/safe spaces.
 - * Higher risk times of the day.
 - * Trauma-informed care principles.
 - * Orientation to the ward to ensure people are aware of behavioural expectations and how to alert staff to risks or to call for help.
 - * Recognising priority groups that are particularly vulnerable to sexual or predatory risk.

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- * Assessment of consumers' capacity to provide sexual consent.
- * vigilance in assessing and managing potential perpetrators of sexual safety incidents.
- Please refer to:
 - * [Chief Psychiatrist's Guidelines for the Sexual Safety of Consumers of Mental Health Services in Western Australia 2020.](#)
 - * [RKPG Sexual Safety in Mental Health Policy and Procedure.](#)

Compliance monitoring

Compliance will be monitored by any clinical incidents via Datix

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References

1. Australian Commission on Safety and Quality in Health Care (ACSQH). The National Safety and Quality Health Service Standards Safety and Quality Improvement Guide: Standard 1 Governance for Safety and Quality in Health Service Organisations. 2nd ed. (Version 2) 2021. Available from <https://www.safetyandquality.gov.au/our-work/assessment-to-the-nsqhs-standards/>

Links

[Australian Government National Standards for Disability Services](#)

[Australian Government National Standards for Mental Health Services](#)

[Chief Psychiatrist's Guidelines for the Sexual Safety of Consumers of Mental Health Services in Western Australia 2020](#)

[DoH Principles and Best Practice for the Care Of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)

[Department Of Health \(DoH\) Principles and Best Practice for the Care Of People Who May Be Suicidal](#)

[Government of Western Australia \(WA\) Mental Health Act 2014](#)

[RKPG Acutely Aroused Patient Clinical Practice Standard \(CPS\)](#)

[RKPG Care of Person Who May Be Suicidal \(Acute\) Policy.](#)

[RKPG Intake Meeting, Case Allocation and Case Review \(Mental Health\) Policy](#)

[RKPG Patient's Property - Management in Mimidi \(Mental Health\) Guideline](#)

[RKPG Restraint and Seclusion: Reducing Restrictive Interventions CPS.](#)

[RKPG Sexual Safety in Mental Health Policy and Procedure.](#)

[South Metropolitan Health Service \(SMHS\) Rights of patients in mental health](#)

[WA Charter of Mental Health Principles MHA 2014](#)

[WA Chief Psychiatrists Standards for Clinical Care](#)

National Standards



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